Pelvic exenteration for men and people assigned male at birth

Pelvic exenteration in men and people assigned male at birth is an operation (surgery) to remove multiple organs in the pelvis. It can be used to treat cancer in the pelvis.

On this page

- About pelvic exenteration
- When is pelvic exenteration used?
- Types of pelvic exenteration
- Sex life after pelvic exenteration
- Fertility after pelvic exenteration
- Your feelings about pelvic exenteration
- Where to get help and support
- About our information
- How we can help

About pelvic exenteration

Pelvic exenteration is a major operation to remove multiple organs in the pelvis. It can be used to treat cancer in the pelvis. The pelvis is the area of the body between the hips, in the lower part of the tummy (abdomen). It contains:

- the sex organs
- the bladder
- a section of the small bowel
- the lower end of the large bowel (colon, rectum and anus).

The pelvis also contains the pelvic bones, lymph nodes (glands), blood vessels and nerves.

For men, trans women and other people assigned male at birth, the sex organs you are born with are the:

- prostate gland
- seminal vesicles
- testicles
- penis.

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Image: The pelvis

When is pelvic exenteration used?

Pelvic exenteration is sometimes used to treat cancer that has spread within the pelvis, or has come back in the pelvis after other treatment.

It may be used to treat:

- rectal cancer
- anal cancer
- bladder cancer.

We have more information about treatment for your type of cancer and what happens before and after the operation.

Types of pelvic exenteration

There are different types of pelvic exenteration:

- total exenteration
- anterior exenteration, this is sometimes called a radical cystectomy
- other exenteration operations.

Your surgeon decides on the type of exenteration you need depending on the results of the tests and scans you have had. They will discuss these at a multi-disciplinary team (MDT) meeting. It also depends on:

- the type of cancer you have
- where the cancer is in the pelvis
- what treatments you have had before.

If you are transgender or non-binary

If you have had genital gender-affirming surgery, you may have additional questions about how this affects the treatment you need. Your doctor or nurse can give you more information about what your operation will involve.

We also have more information for trans and non-binary people about cancer, cancer treatment and finding support, that you may find helpful.

Total exenteration

This surgery involves removing all the organs in the pelvis. The surgeon will remove:

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- the bladder
- the lower part of the bowel (the rectum and sometimes the anus)
- the prostate
- seminal vesicles.

Image: Total exenteration

Anterior exenteration (radical cystectomy)

This type of pelvic exenteration is used for invasive and advanced bladder cancer. It is also called a radical cystectomy. The bladder, prostate gland and seminal vesicles are removed.

Image: Anterior exenteration

Other exenteration operations

Sometimes your surgeon may suggest an operation that involves taking more areas of tissue. For example, as part of your operation they may need to remove:

- muscles
- nerves
- bones
- blood vessels.

If you need more extensive surgery like this, your surgeon will explain what is involved.

If you are having reconstruction

Depending on what type of pelvic exenteration you have, parts of the body may need to be reconstructed. This means having plastic surgery to rebuild parts of the body that have been removed.

To do this the surgeon may use flaps of skin, fat and muscle. These are taken from other parts of the body such as the buttocks, thighs or tummy. These flaps are shaped and stitched into place. This may be done at the same time as your pelvic exenteration operation, or at a later date.

You will have a wound where these flaps are taken from. A nurse will look after this when you are in hospital and will arrange for it to be looked after when you get home.

If the bladder or rectum are removed

If the bladder or rectum are removed during your surgery, the surgeon will make a new way for urine (pee) or stools (poo) to leave the body. Your surgeon or nurse will talk to you about this before surgery. It usually means you will have a stoma after surgery.

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A stoma is an opening that the surgeon makes on your tummy (abdomen). You may have a stoma that:

- allows stools to pass out of the body, called a colostomy
- allows urine to pass out of the body, called a urostomy
- allows urine and stools to pass out of the body, called a Carter stoma or double barrel stoma.

You wear a disposable bag over the stoma to collect the stools or urine.

Sex life after pelvic exenteration

The physical changes to your body after surgery can mean changes to your sex life. The operation will affect you both physically and emotionally. These side effects may happen if a treatment damages tissue, nerves or blood vessels in the pelvic area. These side effects may include:

- changes in sexual sensation
- erection or ejaculation problems
- anal or rectal changes
- loss of sex drive.

Your surgeon and specialist nurse will talk to you about the changes you may have. It can help to talk to them about any questions or worries you have.

We have more information about:

- cancer and your sex life – including tips on talking to your healthcare team and coping with your feelings
- male pelvic changes and sex – this includes erection problems, bladder problems and changes to the anus or rectum
- sex and side effects of cancer treatment – including advice on coping with a stoma
- body image – if you are worried about changes to your appearance.

You may not feel like having sex for a while. If you have a partner, you may both need time to get used to any changes. There are different ways you can show your partner you care about them.

This can include spending time together and showing affection through touching, holding hands or putting an arm around their shoulder. You may also find new ways to share sexual pleasure.

After treatment, you may find that your sex life gradually improves. If you continue to have difficulties, ask your specialist nurse or doctor for advice. They can refer you for more specialised support if needed.

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Fertility after pelvic exenteration

Pelvic exenteration can affect your ability to have children (your fertility). The operation can make getting and keeping an erection difficult. It can also affect your ability to ejaculate.

You can ask to be referred to a fertility clinic before having cancer treatment. You may be able to store sperm which can be used later in fertility treatment.

We have more information about fertility in men.

Your feelings about pelvic exenteration

Having pelvic exenteration surgery can be difficult both physically and emotionally. You may have different feelings like stress, anger, anxiety and fear. There is no right or wrong way to feel. How you feel is individual to you.

It is important that you take time to process how you are feeling. Talking to someone you trust may help. Sometimes it is easier to talk to someone who does not know you. You could ask your cancer doctor or GP to refer you to a counsellor.

You may also find it difficult to cope with needing help from others while you are recovering. These are all normal feelings. They are part of adjusting.

Macmillan is also here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line for free on 0808 808 00 00.
- Chat to our specialists online
- Visit our Online Community to talk to people who have been affected by cancer, share your experience, and ask an expert your questions.

Where to get help and support

As well as the support available from Macmillan, there are other organisations that can offer you practical and emotional support

IA – Ileostomy and Internal Pouch Association

IA offers support to help anyone who has had, or is about to have, their colon removed and has an ileostomy or internal pouch.

Urostomy Association

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Urostomy Association offers support and advice to anyone who has had, or is about to have, surgery resulting in a urostomy. It also offers support for family members and carers.

Bowel Cancer UK

Bowel Cancer UK offers information and support to people affected by bowel cancer.

Anal Cancer Foundation

Anal Cancer Foundation provides support and information for people having treatment for anal cancer, and their carers and family members.

LGBT Foundation

LGBT Foundation provides advice, support and information to people who are gay, lesbian, bisexual or transgender, and their families and friends.

Live Through This

Live Through This run a peer support group for LGBTIQ+ people affected by cancer. This group focuses on LGBTIQ+ experiences of cancer and support, rather than on any one cancer type.

Stonewall

Stonewall provides information and support to the LGBT community.

College of Sexual and Relationship Therapists

College of Sexual and Relationship Therapists has a directory of sex and relationship therapists.

Human Fertilisation and Embryology Authority

Human Fertilisation and Embryology Authority provides free impartial information to people affected by fertility treatment. It has information about choosing a clinic.

Fertility Friends

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Fertility Friends is an online community for people coping with infertility.

Fertility Network UK

Fertility Network UK is an online community for people coping with infertility or adoption.

About our information

References

Below is a sample of the sources used in our pelvic exenteration information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

National Institute for Health and Care Excellence; Colorectal Cancer: NICE guideline 151. January 2020. Available from www.nice.org.uk/guidance/ng151/resources/colorectal-cancer-pdf-66141835244485 (accessed November 2022).

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Reviewers

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Chief Medical Editor, Professor Tim Iveson, Consultant Medical Oncologist.

Our cancer information has been awarded the PIF TICK. Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

Reviewed: 01 March 2023 | Next review: 01 March 2027

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Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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